INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Entrectinib monotherapy · adult

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern

Ongoing

Follow the prescribed daily plan

Regimen source

NCCP 00702

Version 2

Daily oral treatment without a rest week

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Your record and contact card	15
05	Sources and further information	18

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Entrectinib aims to control a cancer with a matching gene fusion. It may help shrink or slow cancer, including some cancer in the brain. Your team explains the treatment aim, uncertainty and alternatives for your exact setting.

ROS1 / TRK TARGETED THERAPY

Entrectinib

Blocks abnormal ROS1 or TRK growth signals

Matching gene fusion identified → Abnormal growth signalling blocked → Cancer growth may slow

Entrectinib inhibits proteins made by selected ROS1 or NTRK gene fusions. Blocking these signals can slow cancer growth. The relevant fusion, tumour setting and previous medicines determine eligibility; ROS1 and NTRK are different indications.

Report heart, thinking/memory, vision and bone symptoms early.

Your treatment setting

ROS1 and NTRK are different

This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

NCCP 00702 v2 · official regimen · EMA Rozlytrek product information · updated 27 March 2025 · HSE managed access protocols · ROS1 and NTRK

How long?

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

NCCP 00702 v2 · official regimen

Evidence depends on the gene fusion and tumour

Separate studies found tumour shrinkage and periods of cancer control in ROS1-positive lung cancer and NTRK fusion-positive tumours, including some brain metastases. These studies generally had no randomised comparison group; they do not establish an individual survival benefit or superiority over other targeted medicines.

These group results cannot predict your outcome; other treatment options, prior medicines and tumour type matter.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and checks over time

Relative milestones, not booked appointments or equal time periods. electrocardiogram (ECG)/ electrolytes are checked after one month; daily dosing has no planned rest week.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00702 v2 · official regimen · EMA Rozlytrek product information · updated 27 March 2025

THE WHOLE TREATMENT JOURNEY

Entrectinib monotherapy · adult: treatment and checks

Your treatment pattern

Daily oral treatment without a rest week

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ROS1-positive advanced NSCLC or separate eligible NTRK fusion-positive tumour

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First month	Ongoing daily treatment	Benefit / toxicity review	Hold or change
Treatment NCCP 00702 · SMPC	Confirm ROS1 or NTRK setting and prescription	Daily capsules without a rest week	Continue prescribed dose if authorised	Review benefit and tolerability	Team authorises hold, restart, switch or stop
Blood tests NCCP 00702 · SMPC	Blood count, kidney/liver tests and uric acid	Repeat at agreed reviews	Continue regular testing	Extra tests for symptoms/ abnormal results	Follow recovery and unresolved effects
Heart checks NCCP 00702 · SMPC	ECG/ electrolytes; left ventricular ejection fraction (heart pumping measurement) (LVEF) if symptoms/ risk	ECG/ electrolytes after one month	Further checks periodically as indicated	Assess new breathlessnes s/swelling/ fainting	Confirm recovery before restarting

Care / check	Before treatment	First month	Ongoing daily treatment	Benefit / toxicity review	Hold or change
Symptoms and supply NCCP 00702 · SMPC	Cognition, vision, neuropathy and medicine list	Review symptoms, adherence and supply	Report thinking/ memory, visual or bone changes	Review interactions and practical support	Continue contact and reproductive precautions
Cancer assessment NCCP 00702 · SMPC	Validated fusion, stage and prior therapy	Clinical review	Scans at Consultant-directed intervals	Discuss response/ progression and preferences	Agree next treatment or follow-up

What each check is for

1 **Confirm the setting**

Before starting

The fusion and previous inhibitor change eligibility.

Ask / **act:** Confirm validated testing, MAP approval, prescription and baseline checks.

2



One-month heart review

After one month

QT and electrolytes can change without symptoms.

Ask / **act:** Agree who books the ECG/blood tests and reviews the results.

3



Ongoing review

During daily therapy

Benefit, symptoms and organ function can change.

Ask / **act:** Keep the medicine list current and report symptoms between visits.

4 **Hold or change**

When toxicity/progression requires

Recovery and restart differ by adverse effect.

Ask / **act:** Follow the written clinician plan; continue follow-up and contraception advice.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Before and during treatment

Regular checks help find changes early.

Before the first dose

Before starting: confirm fusion, previous targeted treatment, access and pregnancy precautions; blood count, kidney/liver tests, uric acid, ECG/electrolytes and cognitive symptoms. Heart-pumping assessment is needed for heart-failure symptoms or risk; brain imaging follows the Consultant plan.

Ongoing reviews

During treatment: repeat blood count, kidney/liver tests and uric acid at agreed reviews. Repeat ECG/electrolytes after one month, then periodically as indicated. Assess thinking/memory, heart symptoms, vision and bone pain; scans follow the Consultant plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Daily capsules and planned reviews

No dates or personal information are needed. Confirm actual appointments with your team.

Treatment or check	Timing in this view
Preparation	**Before treatment** Confirm fusion, indication, access, blood tests and electrocardiogram (ECG).
Daily treatment	**First month → Ongoing daily treatment → Benefit / toxicity review** Prescribed entrectinib every day; no planned rest week.
Heart and blood checks	**First month → Ongoing daily treatment → Benefit / toxicity review** ECG/electrolytes after one month, then periodically as indicated; regular blood and symptom reviews.
Hold or change	**Hold or change** Source-specific recovery and clinician decision before restart.

Continue only as prescribed. A symptom or abnormal test can change treatment at any time.

Columns show sequence, not equal periods; the map does not authorise a restart.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Taking entrectinib

Follow your own prescription.

Whole capsules and missed doses

Take whole capsules with or without food; do not crush or chew. Avoid grapefruit, grapefruit products and Seville oranges. If a dose is missed, take it unless the next dose is due within 12 hours; confirm the exact boundary with your pharmacist. The leaflet permits repeating whole capsules after immediate vomiting: follow your written plan and call if uncertain. Suspension or tube administration requires separate pharmacy instructions.

Interactions

Give the team a complete medicine and supplement list, including St John’s wort. CYP3A inhibitors/inducers and medicines affecting heart rhythm need pharmacy review. Do not change the cancer dose yourself; temporary interaction doses differ from toxicity reductions.

Swallowing difficulty

Ask pharmacy for the approved method. Current SmPC allows capsule suspension and enteral administration; the older NCCP says not to open capsules. Instructions and vomiting advice depend on the formulation.

Living with treatment

Report changes between visits.

Thinking, vision and driving

Do not drive or use machinery while experiencing dizziness, fainting, blurred vision or cognitive effects. Ask family/supporters to report noticeable memory or behaviour changes.

Bones and falls

New focal pain, changed gait or mobility needs prompt review for a fracture; reduce fall hazards where possible.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need highly effective contraception during treatment and for at least five weeks afterwards; add a barrier method if using systemic hormonal contraception. Men with partners who could become pregnant need highly effective contraception during treatment and for at least three months afterwards. Do not breastfeed during treatment. Discuss fertility and family plans before starting.



Support medicines

Use prescribed symptom treatment. No routine steroid, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or antimicrobial prophylaxis is specified for this monotherapy; avoid borrowing an infusion antiemetic schedule.

NCCP 00702 v2 · official regimen · NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 · oral therapy · NCCP NCIS antiemetics V8 · parenteral scope

When to get help

Do not wait for the next appointment.

Call urgently

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit’s temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help.

What studies can tell us

Your decision

Ask how the correct molecular and tumour-specific study population compares with your situation. Study averages do not establish personal prognosis or rank current therapies.

Fan et al. 2024 · ROS1 integrated analysis · Demetri et al. 2022 · adult NTRK integrated analysis · Cho et al. 2024 · NTRK-positive NSCLC analysis

When treatment changes

Continuation and restart are clinical decisions.

Hold or stop

Ask why treatment is held, which tests or symptoms must recover, who will authorise restarting and what follow-up is needed.

Practical support

Symptom medicines

NCCP rates entrectinib minimal-to-low oral emetic risk. Agree individual rescue treatment and check heart-rhythm interactions; no routine infusion premedication is specified.

Daily life and medicines

Keep the medicine list current. Avoid driving or machinery while dizzy, faint, visually affected or cognitively affected. Review falls risk and report bone pain.

Pregnancy and breastfeeding

Use the agreed contraception plan during and after treatment. Do not breastfeed during treatment; ask about fertility and family plans before starting.

Taking tablets / attending treatment

Practical treatment details

Supply and appointments

Arrange repeat prescriptions before capsules run out.

Oral treatment needs no routine infusion line or pump.

Questions to ask

Why do I need an ECG?

Entrectinib can affect heart rhythm. ECG/electrolytes are checked before treatment, after one month and periodically as indicated; new fainting or palpitations needs urgent advice.

Can I drive?

Do not drive or use machinery while dizzy, faint, cognitively affected or visually affected. Ask your team when it is safe to resume.

Do I need to enter dates?

No. The timeline shows relative stages. The team books actual tests and reviews the results.

Explain the plan back: Explain your daily dose, what to do after a missed or vomited dose, which symptoms need a call, and who arranges your one-month ECG/electrolytes.

Adult treatment scope · NCCP 00702a / 00702b This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous entrectinib · individual support

Active medicines: entrectinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP minimal-to-low oral emetic risk: agree individual rescue treatment and check QT/interactions. NCIS V8 parenteral schedules are not a daily oral prescription.
- No routine premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), steroid course or antimicrobial prophylaxis is specified.
- Give the team a complete medicine and supplement list, including St John’s wort. CYP3A inhibitors/inducers and medicines affecting heart rhythm need pharmacy review. Do not change the cancer dose yourself; temporary interaction doses differ from toxicity reductions.
- Take whole capsules with or without food; do not crush or chew. Avoid grapefruit, grapefruit products and Seville oranges. If a dose is missed, take it unless the next dose is due within 12 hours; confirm the exact boundary with your pharmacist. The leaflet permits repeating whole capsules after immediate vomiting: follow your written plan and call if uncertain. Suspension or tube administration requires separate pharmacy instructions.
- Avoid pregnancy. Women who could become pregnant need highly effective contraception during treatment and for at least five weeks afterwards; add a barrier method if using systemic hormonal contraception. Men with partners who could become pregnant need highly effective contraception during treatment and for at least three months afterwards. Do not breastfeed during treatment. Discuss fertility and family plans before starting.
- Review cognition/driving, falls/fractures, uric acid, swelling, visual symptoms and practical adherence needs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Adult treatment scope · National Cancer Control Programme (NCCP) 00702a / 00702b This adult suite covers entrectinib alone for ROS1-positive advanced non-small-cell lung cancer (NSCLC) before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

NCCP 00702 · VERSION 2

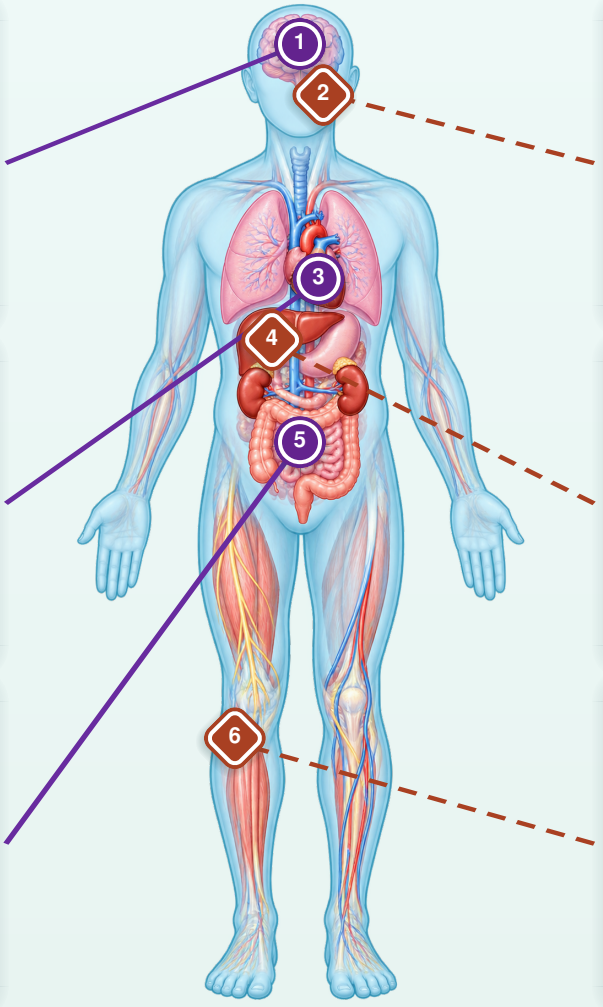
Entrectinib monotherapy · adult

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Dizziness / thinking changes

entrectinib

Report new confusion, memory or balance problems; avoid driving while affected.

2 Visual changes

entrectinib

New blurred vision or other visual disturbance needs review.

3 Breathlessness / swelling / fainting

entrectinib

Heart pumping or rhythm problems require urgent assessment.

4 Jaundice / liver tests

entrectinib

Liver injury may need blood tests and interruption.

5 Bowel symptoms / nausea

entrectinib

Report severe symptoms or dehydration promptly.

6 Bone pain / altered gait

entrectinib

Possible fracture needs prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Dizziness or fainting	☐	☐	☐	
Thinking / memory changes	☐	☐	☐	
Breathlessness or swelling	☐	☐	☐	
Nausea / bowel changes	☐	☐	☐	
Visual disturbance	☐	☐	☐	
Bone / muscle / nerve symptoms	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help. Emergency symptoms: 112 / 999.

Carry your treatment details

Adult treatment scope · NCCP 00702a / 00702b This adult suite covers entrectinib alone for ROS1-positive advanced NSCLC before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

Print, cut out and fold. Follow your treating hospital's contact plan.

SACTCHECK · TREATMENT CARD

Entrectinib monotherapy · adult

NCCP 00702 v2

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital's opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Call the oncology team urgently for new or worsening breathlessness, swelling, palpitations or fainting; marked confusion or memory change; fever, jaundice, severe diarrhoea or inability to drink. Report new visual disturbance and focal bone pain promptly. Follow your unit's temperature/contact instructions. Severe breathing difficulty, chest pain or collapse needs emergency help.

https://sactcheck.com/patient/00702/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00702 v2 · official regimen**
   https://healthservice.hse.ie/documents/6770/702_V2_Entrectinib_monotherapy.pdf
2. **EMA Rozlytrek product information · updated 27 March 2025**
   https://www.ema.europa.eu/en/documents/product-information/rozlytrek-epar-product-information_en.pdf
3. **HSE managed access protocols · ROS1 and NTRK**
   https://healthservice.hse.ie/staff/information-healthcare-workers/medicines-management-programme-mmp/managed-access-protocols/
4. **Fan et al. 2024 · ROS1 integrated analysis**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11733145/
5. **Demetri et al. 2022 · adult NTRK integrated analysis**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC9365368/
6. **Cho et al. 2024 · NTRK-positive non-small-cell lung cancer (NSCLC) analysis**
   https://pubmed.ncbi.nlm.nih.gov/38171156/
7. **NCCP NCIS antiemetics V8 · parenteral scope**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
8. **NCCP systemic anticancer therapy (SACT) antiemetic guidance V6 · oral therapy**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf